جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : عصمت محمد حسين بدر
المحافظة : الجيزه **الحي / المدينة :** العمرانيه **العنوان :** الحريه
رقم البطاقة : 26210130102681 **رقم التليفون :** 01226392214 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 13-10-1962 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عصمت محمد حسين بدر
National ID: 26210130102681

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية



عصمت
محمد حسين بدر
٣١ ش الحرية - العمرانية الغربية
العمرانيه - الجيزه

١٩٦٢/١/١٣

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بدون عمل

أنثى مسلمة متزوجة
سيد عبدالرحمن عبدالرحمن رضوان
١ البطاقة سارية حتى ٢٠٢٨/٠٦/٠٢

National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

جوستاف روسي

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

09/02/2026

MDT FORM

Name:	Ismat Mohamoud Hessen Badr	**MRN:**	6890
Age:	64 Y	**ID:**	26210130102681
Menopausal Sta	postm	**Phone:***	01226392214
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:	dm	**Reason Of MDT:**	triple positive with rd to shift to ht
Surgical History:*	none	**ECOG Performaace Status**	○ 0● 1○ 2○ 3○ 4

Clinical Data:

BX:

ER: **PR:** **HER2:** **KI67:**

History:

POSTMENOP RT BREAST CANCER CT2 NX MX IDC2 ER8/8 PR4/8 HER23+
SM RT UOQ MASS 2 MASSES 10X11MM AT 9 OCLOCK CTS MED LN 10X8
PUL NOD 8X6 MM D3 LYTIC LESION BS FREE
mri dorsal vertebrae 2/2026 multiple dorsal met deposits most app d3,d4 (bs free)
pt rec taxol and herceptin rec 29 weeks taxol and 6 herceptin iv cts on 7/7
rd of rt ax ln newly developped pelvic bone osteolytic lesions for bs rd of subcentimetric pul nodules sd of dorsal lesions
mri dl multiple vertebral mets
bs 8/2026 free
for mdt

R L

TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Sonomammography:

Clinical Staging:

Biomarker/Molecular

Pathological Staging:

Mets Workup:

Recommendations:

MDT Outcome & Plan

Shift to Herceptin iv + Femara
Zometa/3m

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د. نادي حسن	د. أسامة عبدو		

21/9/2026

about:blank

رشيد مفناصبى

NILE SCAN & LABS مركز النيل للأشعة و التحاليل

الفقرات القطنيه بالصفة

Name : Esmat Mohammed Hussein Badr	**Date : 04-07-2026 08:29**
Branch : Dokki	**Medical No. : 270100**
Ref.Physician Prof : Goustav Roussey Egypt	**Ins.Company : التامين الصحي**

DMS
٦٦٨٩.

MRI OF THE DORSOLUMBAR SPINE



TECHNIQUE:

Sagittal T1, STIR and T2 WIs
Axial T1 and T2 WIs
Post contrast T1WIs

FINDINGS:

- Multilevel altered bone marrow lesions are seen affecting multiple of the scanned dorso-lumbar vertebrae being most pronounced at D10, D6, D4 and D3 vertebral bodies. The lesions showed low signal in T1WIs and high signal in T2WIs with post contrast enhancement.
- Maintained dorso-lumbar curve.
- Normal sagittal diameter of the bony dorsal and lumbar spinal canal.
- Spondylodegenerative changes of the lumbar spine evidenced by:
  - Osteophytic lippings of the opposing vertebral endplates
  - Reduced bright T2 signal of the lumbar intervertebral discs
  - Multilevel subchondral fatty marrow degeneration
- L4/5 first degree degenerative spondylolisthesis is seen secondary to bilateral L4/5 facet joint osteoarthritis with L4/5 diffuse posterior pseudo disc bulge is seen effacing the ventral epidural fat and indenting the ventral aspect of the thecal sac with partial encroachment on related neural exit pathways.
- L5/S1 mild diffuse posterior disc bulge is seen effacing the ventral epidural fat, abutting the ventral aspect of the thecal sac and partially encroaching on the related neural exit foramina.
- Normal MRI appearance of the conus medullaris and cauda equina nerve roots.
- No paraspinal lesions seen.

OPINION:

- **Known case of breast cancer showing multiple vertebral lesions, likely metastatic.**
- **Spondylodegenerative changes.**
- **L4/5 first degree degenerative spondylolisthesis with L4/5 diffuse posterior pseudo disc bulge.**
- **L5/S1 mild diffuse posterior disc bulge.**

Much Obliged;
Dr.Marwa Elgawish, MSc, FEBR

NILE DIAGNOSTIC CENTER

Call Center 19656

M.R.I
[[DOCTYPE:MRI]]

روسي

أخصائي متخصص في الصدر والباطنة والحساسية

Patient's Name:	**Esmat Mohammed Hussein Badr**
Date:	**2026-07-07**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

MULTISLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS WITH CONTRAST

TECHNIQUE: Multi slice axial CT images of the chest, abdomen and pelvis were obtained followed by multiplanar reconstruction of the source images. I.V contrast administration.

FINDINGS:

Compared to last study dated 25/12/2025:

CECT CHEST:

- Resolution of the previously seen right breast soft tissue lesion.
- Axial CT shows stationary several direct signs of small airways disease with centrilobular nodules, tree-in-bud opacities, and peripheral centrilobular lucencies consistent with bronchiolectasis.
- Regressive course of the multiple scattered pleural based sub centimetric nodules, the one seen at right lower lung lobe measures 3x4mm compared to 8 x 6 mm. yet largest now is seen at left upper lung lobe measuring 7x6mm.
- Regressive right axillary lymph nodes size, largest one on right side measures 4x3mm compared to 8 x 5 mm.
- Clear left breast and left axilla.
- Stationary multiple enlarged mediastinal lymph nodes, largest one measures 10 x 8 mm.
- No cavitary lesions or bronchiectatic changes.
- No pleural or pericardial collections seen.
- No gross cardiac abnormality.

CECT ABDOMEN AND PELVIS:

- The liver shows chronic parenchymatous changes with no enhancing focal lesions.
- The gall bladder is still markedly distended measures 16.5 x 5.5 cm. Its cavity shows multiple sizable dense stones, the largest one 2.5cm is obstructing its neck. Associated minimal left hepatic lobe IHBRDs.
- Splenomegaly.
- Multiple dilated venous collaterals at splenic hilum.
- Normal CT appearance of pancreas and both kidneys.
- Regular filling of the urinary bladder showing no stones, masses or diverticulae.
- No abdominal or pelvic pathologically enlarged lymph nodes.

CT Scan
[[DOCTYPE:CT_SCAN]]

جوستاف روسي

استصفاء على الصدر والبطن والحوض

Patient's Name:	**Esmat Mohammed Hussein Badr**
Date:	**2026-07-07**
Referred by:	Prof.Dr. **no doctor**
Hospital:	**Goustave Roussey International Egypt**

- No free collection.
- Small hiatus hernia.

THE SCANNED SKELETON:

- The scanned skeleton shows lytic lesion of DV3.
- Spondylodegenerative changes of the examined spine.
- Anterior ligament ossification of the lower dorsal vertebrae.
- **Newly developed few pelvic bones lytic lesions.**

Opinion:

- **Resolution of right breast soft tissue lesion.**
- **Still noted direct signs of small airways disease with centrilobular nodules, tree-in-bud opacities, and peripheral centrilobular lucencies consistent with bronchiolectasis.**
- **Regressive course as regards the multiple scattered pleural based sub centimetric pulmonary nodules.**
- **Lytic lesion of DV3.**
- **No hepatic lesions.**
- **Small hiatus hernia.**
- **Regressive size of enlarged axillary lymph nodes.**
- **Stationary size of enlarged mediastinal lymph nodes.**
- **Newly developed few pelvic bones lytic lesions.**

  Stationary other findings:

  - **The gall bladder is markedly distended. Its cavity shows multiple sizable dense stones, the largest one is obstructing its neck. Associated minimal left hepatic lobe IHBRDs.**
  - **Splenomegaly.**
  - **Multiple dilated venous collaterals at splenic hilum.**
  - **Anterior ligament ossification of the lower dorsal vertebrae.**

Dr. Mai Shaaban MD

M.R.I
[[DOCTYPE:MRI]]

GUSTAVE ROUSSY INTERNATIONAL

وستاف روسي

ECHO REPORT

NAME: [illegible] DATE: 20/1/2021

ID NUMBER: ٢٤٤١/٢٠ SEX: ♀ AGE: 62

EF	66%	55-75%	PWT	1.2	0.6-1.1cm
LVEDD	4.8	3.5-5.8cm	AO.R	3.0	3-3.7cm
LVESD	3.0	2-4cm	LA	3.8	2-4cm
FS		25-45%	RV		0.7-2.3com(supine)
SWT	1.2	0.6-1.1cm	AO.CUSP		1.5-2.6cm

LV Dimensions

LV Wall Thickness

LV Diastolic Function DDI

SWMA No

LA and Ao Root Dimensions

Mitral Valve

Aortic Valve

Tricuspid Valve

Pulmonary Valve MildTR

Right Side and Systolic Function TRSP=20

Intracardiac Masses and Effusion

DR.

Echo
[[DOCTYPE:ECHO]]

Name : Esmat Mohammed Hussein Badr
Date : 22-08-2026 10:18
Medical No. : 444945
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

63 year-old female patient, who gave history of treated right breast cancer and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- Considering the last bone scan report dated 10/12/2025, the current scan revealed uniform radiotracer uptake is noted allover the surveyed skeleton with no evidence of focal active or cold osseous lesion that would account for bone secondaries.
- Degenerative features are depicted at different levels of the spine.
- Osteoarthritic changes are noticed around big joints.

IMPRESSION:-

- The skeleton is keeping free from osseous metastasis.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

NILE DIAGNOSTIC CENTER

Bone Scan
[[DOCTYPE:BONE_SCAN]]

جوستافروسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

01/13/2026

MDT FORM

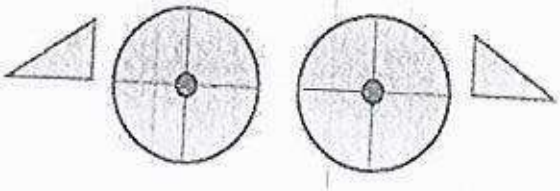
Name:	Ismat Mohamoud Hessen Bedr	MRN:	6890
Age:	63 Y	ID:	26210130102681
Menopausal Sta	post	Phone:	01226392214
NRS SCORE:		Marital Status:	married
BMI:		Family History:	
CO Morbidity:	HTN	Reason Of MDT:	NEW

Clinical Data:

BX: IDC2

ER: 8/8 PR: 4/8 HER2: 3+ KI67: 15%

R L



Sonomammography:
UOQ masses 2 masses + indeterminate LN

Mets Workup:
ct cap d3 lytic lesion
bone scan free

Recommendations:

MDT Outcome & Plan
FOR CLIPPING OF 2 MASSES, FOR THE POSSIBILITY THAt is not metastatic
Biopsy from D3 lesion
Start taxotere / Herceptin]

TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ ملك عبدالعزيز	د/ محمد الحروني				

STAVE/
USSY
ATIONAL

Patient Name	: الأستاذه / عصمت محمد حسين بدر	Request No.	: 13001023
Patient No.	: 26210130102681	Request Date	: 03/11/2025
Path No.	: 3359/25	Verification Date	: 11/11/2025
Sex	: Female	Financial Category	: جوسكاف رومي - مباشره
Age	: 63 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast mass.
Nature of Specimen: Tru-cut biopsy.

Gross Examination:
- Five grey white tissue cores, the longest was 1.5 cm. Totally submitted.

Technique:
- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5) & Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* DCIS: Focal intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).
* Microcalcification: Present.
* Angioinvasion: Absent.
* Perineural invasion: Absent.

* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 95% of tumor cells (score 8/8).
- PR: Moderate nuclear reaction in 10% of tumor cells (score 4/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 15%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.
- Focal Intermediate grade ductal carcinoma insitu (Solid & cribriform patterns).

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

USTAVE/
OUSSY

Patient Name	: الأستاذه / عصمت محمد حسين بدر	Request No.	: 13001023
Patient No.	: 26210130102681	Request Date	: 03/11/2025
Path No.	: 3359/25	Verification Date	: 11/11/2025
Sex	: Female	Financial Category	: جرستاف روسي - مباشره
Age	: 63 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
- ER: Positive (score 8/8).
- PR: Positive (score 4/8).
- Her2: Positive (score 3+).
- Ki67%: 15%.

* Scoring system; "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahem Ghoneim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / عصمت محمد حسين بدر	Request No.	: 13001023
Patient No.	: 26210130102681	Request Date	: 03/11/2025
Path No.	: 3359/25	Verification Date	: 11/11/2025
Sex	: Female	Financial Category	: جوستاف روسي - مباشره
Age	: 63 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Verified by :
Dr.Duaa Mohamed

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient ID: 89873	Patient Name English: Esmat Mohamed Hussein Badr
Age: ٢٢	2025/11/03 10:23:28 AM :Report Date

Women's Health Outreach Program
Bilateral full field Digital Mammography

Clinical summary: right breast lump

Technique

Medio-lateral oblique and CC views & High resolution ultrasound

Detailed Findings

Right breast

- Scattered fibroglandular densities (ACR B)
- UOQ two areas of distortion with internal calcifications seen about 2 cm from each other and about 6.5 cm from the nipple, target us showed corresponding two palpable areas of parenchymal distortion seen at
  - 9:10 o'clock (zone b) measures 10x11mm
  - 9 o'clock (zone b) measures 14x15mm
- no cystic lesions.
- Normal skin and nipple/areolar complex.
- Single indeterminate axillary LN (level I) with preserved hilum and increased cortical thickness reaching 3.5mm.

Left breast

- Scattered fibroglandular densities (ACR B)
- No masses, asymmetric densities, distortion or suspicious clusters of microcalcifications.
- Ultrasound demonstrated no solid or cystic lesions.
- Normal skin and nipple/areolar complex.
- Non-specific axillary LNs.

Impression:

Right breast (BIRADS 4c): UOQ suspicious two palpable areas of parenchymal distortion, with ipsilateral single indeterminate axillary lymph nodes

Left breast (BI-RADS 1) : normal study.

Best regards,
Dr. Sarah Gamal ,MSc

Mammogram
[[DOCTYPE:MAMMOGRAM]]